

History: Chronic pain right elbow and shoulder.

MRI RIGHT SHOULDER

Technique: Multiparametric MRI right shoulder.

Findings: The rotator cuff is intact. Rotator cuff muscle bulk is normal.

Long head biceps tendon is intact. There is hypointense tissue within the inferior glenohumeral joint space (see key image). A suspected tear of the anteroinferior labrum is present with poorly defined labrum which demonstrates increased signal. The tissue is most likely a flipped labral fragment. There is a further tear involving the posterosuperior labrum which is attenuated and of increased signal.

Glenohumeral cartilage is maintained. AC joint appears normal. Trace increased fluid within the subacromial/subdeltoid bursa.

Conclusion: Suspected anteroinferior labral tear with displaced labral tissue in the inferior glenohumeral joint. Further small chronic appearing posterosuperior labral tear is present.

MRI RIGHT ELBOW

Technique: Multiparametric MRI right elbow.

Findings: Low grade tendinosis of the common extensor origin with a very small intrasubstance tear involving the deepest fibres. Tear cleft is very linear measuring approximately 3mm in length. The radial collateral ligament is poorly defined towards the humeral attachment but thought to be grossly intact. Collateral ulnar collateral ligament is intact.

Common flex origin and ulnar collateral ligament is intact.

Biceps, triceps and brachialis appear satisfactory.

Articular cartilage of the elbow is maintained. No effusion. No loose bodies.

Normal appearance of the median, radial and ulnar nerve.

Conclusion: Low grade tendinosis of the common extensor origin with minor intrasubstance tearing.